

Thyroid problems

» What are they?

The thyroid gland regulates the body's metabolism. Disorders occur if it produces more thyroid hormones than the body needs, or not enough.

» Risks to your pregnancy, labor, and baby

Untreated hyperthyroidism (overactive thyroid) or hypothyroidism (underactive thyroid) can pose serious problems in pregnancy. Thyroid hormones play a key role in developing your baby, so your doctors will try to keep your condition under control and avoid the risks of preeclampsia, heart failure, kidney failure, coma, premature labor, stillbirth, or miscarriage. Preconception planning is very important to optimize your control of the condition as some studies suggest that a very underactive thyroid in the first trimester may affect a baby's IQ and increase the risk of learning difficulties.

» Medicines and tests

Your doctor may review your medication and possibly advise increased dosage if you need it. Hypothyroidism is sometimes treated with the synthetic hormone thyroxine during pregnancy. Your thyroid function will be checked regularly throughout your pregnancy using blood tests. You will have additional ultrasounds to check the health of your baby during your third trimester.

» Effects of pregnancy on your condition

Women with an overactive thyroid that is stable may have a relapse since pregnancy hormones can increase the circulating levels of thyroid hormone in the blood. Pregnancy can cause swelling in the thyroid gland, called a goiter.

Rheumatoid arthritis

» What is it?

This long-term inflammatory disease causes pain, swelling, and stiffness or immobility in the joints.

» Risks to your pregnancy, labor, and baby

Other than a possibility of premature labor, rheumatoid arthritis (RA) doesn't usually directly affect your pregnancy or baby. If it affects your hips or pelvis, you may need to have a cesarean section.

» Medicines and tests

Some RA medications are not safe for the baby. Others are safe, but can cause raised blood glucose or high blood pressure for you. Talk to your doctor as soon as you discover that you are pregnant (or planning a pregnancy) to discuss your medication.

» Effects of pregnancy on your condition

Pregnancy can increase symptoms of fatigue, especially in the first trimester. In the second trimester, research suggests that in up to 70 percent of women with RA symptoms may improve, with these improvements lasting for the first few weeks after pregnancy. However, some women experience a flare-up after this period.

Diabetes

» What is it?

Diabetes is a condition in which the body is unable to control blood glucose levels, either because it produces too little insulin, or because it cannot use the insulin it produces (see box, right). Diabetes in pregnancy can be preexisting or can develop in pregnancy (gestational diabetes, see p.145). Here we discuss preexisting diabetes.

» Risks to your pregnancy, labor, and baby

Diabetes increases your risk of high blood pressure and preeclampsia (see p.144), and of having a baby with birth defects. It also increases your risk of miscarriage, stillbirth, and preterm labor. High glucose levels in your blood mean that glucose in your baby's blood also increases, and your baby may be large at birth. Babies born to diabetic mothers may also be small at birth.

» Medicines and tests

It is very important to optimize your control of diabetes pre-pregnancy to minimize the chances of birth defects. It is also essential to continue to manage glucose levels once pregnant. Talk to your doctor as soon as you know that you're pregnant for a referral to an endocrinologist or a diabetes specialist. He or she will give you glucose targets to maintain in pregnancy. If you are on synthetic medicines you may switch to insulin injections, or you may continue with your oral medicine. You will need to follow a glucose-friendly diet; a dietician will work out a meal plan with you. Your doctor will encourage you to test your blood glucose levels at least four times a day to make sure they remain on target. As your pregnancy progresses, you'll probably have to increase the amount of insulin you inject to keep glucose levels stable. Make sure that your friends and family know how to use the testing kits, too, just in case you have a hypo or hyper episode (both of which can lead to unconsciousness) and aren't able to test for yourself. Throughout your pregnancy, you might be offered additional growth ultrasounds to make sure your baby is growing appropriately, remembering that a large baby may indicate that your blood glucose levels are too high.

» Effects of pregnancy on your condition

Pregnancy hormones increase insulin resistance; that is, they exacerbate your body's inability to use insulin to regulate blood glucose levels. As a result, you are at a greater risk of all the complications of diabetes—hypo- and hyperglycemia, eye problems, heart disease, and kidney disease, and you'll be monitored closely for these. If you suffer from vomiting, the concentration of acids called ketones may increase in your blood. Ketones are a by-product of burning fat reserves for energy, rather than glucose. Excessively high levels can lead to serious

pregnancy complications, and may be life-threatening for you. This situation is extremely rare, and most women with diabetes have healthy pregnancies, but managing your glucose levels through your diet and insulin injections is of utmost importance.

DIABETES FACTS

Here are the important facts about preexisting diabetes and the signs to look for if haven't already been diagnosed.

» **Your body derives glucose** from the food you eat, in particular from carbohydrates, fats, and proteins in your diet.

» **The symptoms of diabetes** include having a dry mouth, frequently needing to urinate, fatigue, blurred vision, and yeast—almost all of which are also perfectly healthy symptoms of pregnancy!

» **Type I diabetes** occurs when your body makes too little of the hormone insulin, which it needs to take glucose from your bloodstream and turn it into energy.

» **Type II diabetes** occurs when your body makes enough insulin, but its cells become resistant to the hormone's attempts to draw glucose from them. It is often caused by obesity.



Insulin injections Your insulin requirements will change throughout your pregnancy. It's crucial that you keep your blood sugar under control.